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PSYCHOLOGICAL EVALUATION: **CONFIDENTIAL**

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Respondent Name: David Garibotto Nimer

Age at the time of Evaluation: 33

Date of Evaluation: 05/20/2025

Date of Report: 07/18/2025

Referred by: Arizona Supreme Court, Committee on Character and Fitness

Date of Birth: 5/31/1991

Sex: Male

Primary Language: English

Examiner: Emily T. Bashah, Psy.D.

Reason for Referral:

The Arizona Supreme Court, Committee on Character and Fitness, referred Mr. Nimer for a psychological evaluation. Mr. Nimer is an applicant by examination. This referral is made as a part of the Committee's continuing investigation as to whether Mr. Nimer meets the standards for admission to the practice of law in Arizona pursuant to the Supreme Court Rule 36(b).

Evaluation Process:

Mr. Nimer was interviewed and evaluated in a private office at Bashah Psychological Services, PLC, by Dr. Bashah on 05/20/2025. Mr. Nimer transported himself and arrived on time for his appointment. His identity was confirmed through his driver's license. During the clinical interview, Mr. Nimer's mental status was evaluated, and social, clinical, psychiatric, and substance use histories were obtained. Following the clinical interview, Mr. Nimer was administered an intelligence scale, a personality inventory, and a substance use inventory. Combined, the testing and clinical interview lasted 6 hours and 40 minutes.

Informed Consent Process and Limits of Confidentiality:

The Arizona Supreme Court, Committee on Character and Fitness informed Mr. Nimer of the recommendation for a psychological evaluation and he voluntarily consented to the evaluation. He was thoroughly apprised of the reason for the evaluation, the tests that would be administered, and their purpose. He was also informed of the limitations to confidentiality due to the disclosure of information that would indicate a danger to himself or others and of our record-keeping policies, which conform to state and federal guidelines. He was also advised that the protection of confidentiality does not apply to the evaluation, as anything he disclosed could be used in a written report to be submitted to the Arizona State Supreme Court, Committee on Character and Fitness. With his expressed and written consent, he acknowledged that he understood the information and agreed to participate and proceed with the psychological evaluation. Mr. Nimer authorized the release of the report to the Arizona Supreme Court, Committee on Character and Fitness. Naomi Lopez Felix, B.S., a clinical psychology doctoral trainee, assisted with the evaluation under Dr. Bashah's supervision. It is important to note that Dr. Bashah can attest that the contents of this report are the expert opinions and finalized work product of Dr. Bashah.

This evaluation was conducted in accordance with the Ethical Principles of Psychologists and Code of Conduct of the American Psychological Association, as well as the Specialty Guidelines for Forensic Psychology. All data were gathered and interpreted using scientifically grounded methods, and findings are presented in a manner consistent with accepted standards for forensic psychological practice. The evaluator has maintained professional neutrality and objectivity throughout, with attention to relevant cultural, contextual, and legal factors affecting the examinee and referral question.

Collateral Sources of Information:

The Committee on Character and Fitness, provided the following records for review:

Arizona Supreme Court, Committee on Character and Fitness, Referral Letter	dated 04/10/25
Character and Fitness Application for Admission, NCBE No: N10685417	dated 02/05/25
Boulder County Court, Colorado, Order to Seal Criminal Justice Records, Case No: 16T10	dated 10/24/17
Arizona Supreme Court, Attorney Admissions, Reference Forms	dated 02/06/25-03/13/25
Boulder County Court, Colorado, DUI Legal History, Case No: 15-14751, Case No: 15T2992	dated 11/14/15- 08/31/17
Boulder County Court, Colorado, DWAI Legal History, Case No: 15-12043, Case No: 15T2454	dated 09/17/15- 06/06/16
Arizona Supreme Court, Committee on Character and Fitness, Investigative Notes	undated

Mr. Nimer provided the following medical records, received on 06/06/25:

J. Michael Kerrigan, PH.D., Clinical Psychology and Neuropsychology, Neuropsychological Evaluation	dated 07/04/18
Menninger Comprehensive Psychiatric Assessment Service, Report of Neuropsychological Evaluation by Dr. Bradshaw	dated 02/02/16

Associated Neurologist of BCH, Progress Note
Boulder Community Hospital, Diagnostic Imaging, MRI Report
Boulder Community Hospital, Diagnostic Imaging, MRI Report
CT Brain WO Contrast Findings and Impressions
MRI Brain W WO INF Findings and Impressions

dated 12/04/15
dated 12/02/15
dated 03/12/15
dated 02/08/15
dated 02/08/15

Test Administered:

Wechsler Adult Intelligence Scale-Fifth Edition (WAIS-V)
Personality Assessment Inventory (PAI)
Adult Substance Abuse Subtle Screening Inventory – 4th Edition (Adult SASSI-4)

BACKGROUND INFORMATION

According to the Arizona Supreme Court, Committee on Character and Fitness, investigative notes and the Character and Fitness Application, dated 02/05/25, Mr. Nimer self-disclosed that he sustained a TBI in a motor vehicle accident in 2015 and 3 DUIs from later in 2015. He disclosed that he was “intubated, airlifted, and hospitalized for several days, and following which I dealt with trouble sleeping, headaches, dizziness, and other symptoms for some while. In hindsight, though it is absolutely no excuse for my driving choices, my judgement was also compromised.”

Abbreviated Life History:

Mr. Nimer reported that he was born in San Francisco, California. He stated that he met his developmental milestones within normal limits. He explained that he has a younger brother and an older fraternal twin brother. He described that he had a “rich” childhood as his parents were happily married, he had “lots” of family, played sports, and vacationed often with his family. Mr. Nimer stated that he lived in the “Bay Area” until age 18, in Pennsylvania from ages 19 to 21, in Colorado from ages 21 to 26, and in North Carolina from age 26 to approximately age 28. He added that he moved to Arizona in 2020.

Mr. Nimer stated that he is single, has never been married, and has no children. Mr. Nimer stated that he was living in a 4-bedroom apartment with 3 roommates at the time of this evaluation. He disclosed that his parents “support [him] here and there.” Mr. Nimer reported that the relationship with his family had its “ups and downs.” He stated that his parents had high expectations for him, and his criminal history caused challenges in their relationship.

Education:

Mr. Nimer reported that after he graduated high school, he attended Dickinson College in Pennsylvania. He stated that he transferred to the University of Colorado after attending Dickinson College for 2.5 years. Mr. Nimer explained that his freshman year in Dickinson College, he was placed on stayed suspension for one year due to cannabis possession. Mr. Nimer disclosed that he was suspended in October 2011 from Dickinson College his Fall semester of sophomore year for another possession of cannabis related incident. Mr. Nimer obtained his Bachelor of Arts in Political Science in 2015 from the University of Colorado. He reported that

in 2020 he obtained a master's degree in environmental management from the Nicholas School of the Environment in Duke University. He stated that he graduated from the Arizona State University (ASU) law school in 2023. He recalled that throughout his post-secondary education he struggled academically, with a low GPA, and was placed on academic probation. He elaborated that during law school, he was "kicked out" of the program, had to file an appeal, reapply to the program, and was readmitted during the Spring semester of his second year.

Employment:

Mr. Nimer reported that he worked in a variety of jobs, as an umpire, lifeguard, in the food service industry, housecleaning and carpet care, sales, for a moving company, and construction. Mr. Nimer stated that after he obtained his master's degree, he worked as a paid researcher, pizza delivery driver, and research analyst. He reported that while attending law school, he had legal internships during the summers. He elaborated that he had an internship for the Phoenix Public Defender's Office during the Spring 2022 semester and for an Arizona State University Law Center during the Summer in 2022. He added that he was unemployed for one year while he studied for the bar examination. He recalled that he failed his bar examination in July 2023 and passed his second attempt in February 2024. Mr. Nimer stated that he has been employed at a personal injury law firm since May 2024. He explained that his duties included doing research, writing motions, and writing memos.

Substance Use and Treatment:

Mr. Nimer reported that he first drank alcohol as a child during the Jewish Sabbath. He during his last semester of high school, he drank approximately 5 drinks of liquor and beer once a month. He recalled in college he drank alcohol once to twice a week. He estimated that he drank 5 to 10 drinks of mostly liquor and some beer. Mr. Nimer stated that he blacked out due to alcohol intake a "handful" of times. He described that while he attended Duke University, he would consume 5 alcoholic drinks once every 2 to 3 weeks. He added that during law school he would have 5 drinks once every 3 weeks. He reported that he would drink more heavily once every few weeks. Mr. Nimer stated that he last drank alcohol a few weeks prior to the exam. He estimated that he had 7 drinks of mostly liquor and some beer. He reported that he drinks alcohol once a month due to having to work on the weekends. He expressed that he would "prefer" to drink every weekend but is limited due to his finances.

Mr. Nimer reported that he first smoked cannabis at age 18 when he was a senior in high school. He stated that he smoked cannabis a "handful" of times during the Spring semester of his senior year of high school and during that following summer. He recalled that he smoked most days when he was in college and estimated that he smoked a half ounce of cannabis a month. He added that he smoked a half gram to a gram of cannabis a day. Mr. Nimer reported that he smoked a couple times a day most days during his freshman year of college, 2 to 3 times a week his junior year of college, and once a day during the weekend his senior year of college. Mr. Nimer stated that he was "winding down" on his cannabis and alcohol use while he was on probation in 2018 as he was being drug tested. He recalled that after he completed his probation, he smoked cannabis a few times in North Carolina. He estimated that he smoked approximately once a month, a gram of cannabis a month, and would go a few months without smoking cannabis. He stated that since he moved to Arizona, he smokes a gram of cannabis or less a

month. He recalled that he smoked cannabis a “handful” of times last year and last used cannabis during the Fall of 2024. Mr. Nimer disclosed that he would “like” to smoke more cannabis, once on the weekend preferably, identifying it as his drug of choice. He remarked, “If I could be back to smoking once a week, that’s when I know I’m in a good place,” further indicating that he is likely then satisfied with his job, finances, and goals. He commented, “I’ll go back to it when time is right,” referencing using cannabis. He justified cannabis use for relaxation purposes, stating, “It is good to relax and have fun on the weekends,” as he is “a little tense right now,” by not achieving his potential in his career.

Mr. Nimer initially expressed hesitancy to disclose his use of other substances, but then acknowledged that 10 years ago, while in college, he was “dabbling” in cocaine “lightly.” He denied other substance use. When asked about treatment, Mr. Nimer stated that the only substance use treatment he participated in was mandated. *According to the Character and Fitness Application, he explained that he participated in level 2 alcohol classes, substance monitoring, and attended Mothers Against Drunk Drivers. He stated that he successfully completed two years of probation, released on 08/31/2017.* He assessed that he “didn’t learn much,” aside from deterrence, stating, “there are consequences” as he was impulsive and that he needs to think before he acts.

Legal History:

Mr. Nimer denied having a juvenile criminal history. When asked about adult criminal history, he reported three charges. He disclosed that on September 17, 2015, he was charged with a driving while ability impaired (DWAI). He explained that he was pulled over because he did not stop for a stop sign on his way to a party. He stated that his blood alcohol content (BAC) was 0.055 and spent the night in jail. Mr. Nimer reported that he later charged with driving under the influence (DUI) on November 7, 2015, in Colorado. He stated that he was drinking alcohol at home and called an Uber to take him half a mile away. He recalled that the Uber was delayed, and he decided to drive instead. He remarked that he “should have walked” and that he was being impulsive. He described that while he was driving, his roommate threw a beer can out of the car window and a police officer pulled him over. Mr. Nimer reported that his BAC was 0.1 and he spent the night in jail. He stated the charge was reduced to a DWAI, had to complete 6 months of probation, and “voluntarily” drug tested and stopped drinking alcohol. He noted that the “seriousness” of the charge did not “sink in.” Mr. Nimer reported that he was charged with a driving under the influence of drugs on January 3, 2016. He elaborated that he was pulled over driving home from his friend’s house after smoking cannabis. He stated his charge was dismissed and sealed. He discussed that he intended on joining the army prior to the charges, and that he became ineligible to enlist after his third offense. Mr. Nimer reported that he was on 2 years of probation and completed it “early” in 2017. He specified that he was “perfect” while in monitoring. He reported that he was also sentenced to 4 months of work release.

Psychiatric History:

Mr. Nimer reported that he was diagnosed with attention-deficit/hyperactivity disorder (ADHD) in 2014, at age 21 or 22, and was prescribed a few different stimulants since then.

He recalled that he was prescribed Adderall¹, Concerta², and Vyvanse³. He stated that he was initially hesitant to take his medication but did take it most days of the week when he was studying or working. He added that the medication made him feel like a “zombie” and “more intense.” Mr. Nimer stated that he was “smart,” had “masked” his ADHD prior to his diagnosis, and did well on standardized tests. He explained that he had mental inefficiency that his mother had observed and inattention difficulties. He recalled that during college, he was away from the structure and oversight of his parents and had difficulties that led to academic probation. He elaborated that he was disorganized and undisciplined. Mr. Nimer stated that at ASU law school, he had accommodations such as having additional time to complete exams and a notetaker. He added that during law school he became a better student and more organized. He reported that ADHD is a disability that “continues to drag [him] down” and that he has to put habits in place.

Mr. Nimer reported a history of sleep difficulties since he was a child. He recalled that in college, he would stay up all night and miss his morning classes, he would be tired “all the time,” was a night owl, and had sleep bags under his eyes. He estimated that he would stay up all night twice a week. Mr. Nimer elaborated that he would stay up later and later every day until he was up all night. He added that he was tired and fell asleep while driving which led to a motor vehicle accident in 2015. He reported that he was diagnosed with a circadian rhythm disorder by a sleep specialist. Additionally, Mr. Nimer reported that he participated in neuropsychological testing following his motor vehicle accident, and again in 2018.

When asked about his current symptoms, Mr. Nimer expressed dissatisfaction with his life circumstances. He elaborated that he was not happy with his work, supervisors, social life, or relationship status. Mr. Nimer stated that he was having difficulties searching for a new job and that he would prefer to work in environmental law. He explained that looking for other positions requires sustained focus and that he is not efficient. He added that he was considering returning to California to practice law to be closer in proximity to his family. He reported that at the time of the evaluation he was prescribed 10 mg of Focalin⁴ that he takes as needed and estimated he takes it 3 times per week. He added that he takes liquid melatonin, wears blue light blockers when he is using his laptop, and developed better sleep hygiene habits.

When asked about family history of mental health, Mr. Nimer stated that his maternal aunt was diagnosed with schizophrenia. He denied having a history of psychiatric hospitalizations, suicide attempts, or perceptual disturbances.

¹ Dextroamphetamine-amphetamine (brand name: Adderall): According to the National Library of Medicine, dextroamphetamine-amphetamine is a central nervous system stimulant used to treat ADHD. Retrieved from: <https://www.ncbi.nlm.nih.gov/books/NBK507808/>

² Methylphenidate hydrochloride (brand name: Concerta): According to the National Library of Medicine: methylphenidate hydrochloride is a central nervous system stimulant used to treat ADHD. Retrieved from: <https://pubchem.ncbi.nlm.nih.gov/compound/Methylphenidate-Hydrochloride#section=Effects-During-Pregnancy-and-Lactation>

³ Lisdexamfetamine dimesylate (brand name: Vyvanse): According to the National Library of Medicine, lisdexamfetamine dimesylate is a prodrug stimulant used to treat ADHD. Retrieved from: <https://pmc.ncbi.nlm.nih.gov/articles/PMC2873712/>

⁴ Dexmethylphenidate (brand name: Focalin): According to the National Library of Medicine, dexmethylphenidate is a central nervous system stimulant used to treat ADHD. Retrieved from: <https://pubchem.ncbi.nlm.nih.gov/compound/Dexmethylphenidate>

Medical and Neurological:

When asked about his medical history, Mr. Nimer reported that in high school he played football which led to a concussion and the loss of consciousness a “few times.” He stated that he has lost consciousness a few seconds in a “few” car accidents. He added that he tore his shoulder at age 26, the day he completed probation, and was unable to join the army. He denied a history of seizures.

Mr. Nimer reported that he was in a motor vehicle accident during his senior year of college. He explained that he drove 12 hours from Colorado to Oklahoma to visit his friend. He stated that 10 minutes before his car accident, he had been sleeping in a church parking lot and was approached by law enforcement. He disclosed that he fell asleep while driving and drove off the road where he crashed into a tree. He explained that he lost consciousness for a few days and was induced into a coma. He reported that the accident resulted in his right arm being broken and diagnosed with a mild traumatic brain injury (TBI). Mr. Nimer reported that he “almost died.” He explained that his TBI symptoms included difficulty sleeping, memory loss, repeating himself throughout the day, decreased libido, increased irritability, “less control” of his mood, indecisiveness, and challenges with his short-term memory. He recognized that he was not his “vibrant intelligent self.” He stated that he dropped a class following his accident because of cognitive difficulties. He explained that his three legal infractions related to driving impairments and substance use, occurred after his TBI and that his defense attorney argued that his decision making was therefore impaired.

According to the most recent neuropsychological evaluation by Dr. Kerrigan, Ph.D. dated 07/04/2018, since Mr. Nimer’s motor vehicle accident on 02/07/2015 in which he “sustained a mild-complicated Traumatic Brain Injury (TBI),” he continued to experience some deficits in executive functioning and ongoing impairments related to ADHD, predominant hyperactive presentation. Dr. Kerrigan explained that Mr. Nimer’s executive function difficulties involve: “self-regulation, self-management, self-monitoring and task completion.” Test data further suggests that Mr. Nimer has difficulties with “mental flexibility.” Dr. Kerrigan explains in his report that these executive function deficits are “commonly seen in cases where head trauma has involved blows to the front or back of the head,” and “quite important in achieving academic success, as well as vocational stability and satisfaction.” Dr. Kerrigan added, “While he demonstrated impairment of certain specific skills during this evaluation, his overall level of neuropsychological functioning, as reflected in his Neuropsychological Deficit Score (NDS), is below the threshold indicating Mild Neuropsychological Impairment and falls within the ‘Normal Range’.”

According to the Boulder Community Hospital, Diagnostic Imaging MRI report by Dr. Rao, MD, dated 12/02/2015, Mr. Nimer was diagnosed with Post concussion syndrome. In a progress note by Dr. Rao, dated 12/04/2015, Mr. Nimer’s symptoms reportedly improved since his accident but the diagnosis for Post concussion syndrome was retained. It was reported that his repeat MRI brain imaging revealed an abnormal finding for “some minimal punctate Foci of hemosiderin deposition consistent with his previous axonal injury/TBI.”

Mental Status/Behavioral Observations:

Mr. Nimer is a 33-year-old, White male. He reported that his mother is Italian and English, and that his father is of Jewish Slavic origin. When asked about his spiritual affiliation,

he stated that his parents are interfaith, and he celebrates both Christian and Jewish holidays. He is right-handed with a non-inverted writing posture. He reported that he is 6'3" and weighs approximately 190 pounds. He was clean, dressed casually, and has a fair complexion. He had straight, short black hair; dark brown eyes; and short facial hair. When asked about his hearing, he reported that he had below average hearing, but it did not appear to interfere with the current evaluation. When asked about his vision, he reported that he had Lasik surgery on his left eye and does not wear glasses. No vision or hearing difficulties were observed. When asked about tattoos and scars, he did not report any tattoos but stated he had scars on both of his shoulders from surgeries due to sports injuries; keloids scars on his arms and legs he obtained from broken glass due to car accidents; and a scar on his left arm he obtained in college installing windows.

Mr. Nimer presented as cooperative, though he appeared to be self-monitoring for impression management. It was noted that when asked directly about any school suspensions, he neglected to disclose his history and the examiner needed to ask about specific incidents noted in the records provided to obtain the information regarding his school suspensions. He appeared irritable when the examiner probed about the frequency and quantity of his substance use, and when he was interrupted or redirected by the examiner. Mr. Nimer was fully oriented. He demonstrated some perseveration and had to complete his thoughts before switching focus to another topic. His thought process was tangential at times, circumstantial, and required redirection from the examiner. It was noted that he exhibited difficulty redirecting and, as noted above, had to complete his thought before moving on. His eye contact was often avoidant. His speech was within normal limits, but he was observed to mumble at times. Mr. Nimer was observed to organize his personal items repeatedly and aligned his items at the start of the evaluation. Towards the end of the evaluation, he became more informal as he was observed to remove his shoes and lay down fully on the couch. When asked about his current mood, he stated he felt tense, stressed, dissatisfied with his job and life circumstances, and impatient with his current timeline. His affect appeared depressed, frustrated, irritable, and unhappy.

Overall, Mr. Nimer displayed mental inflexibility when being redirected and was preservative. He demonstrated compensatory strategies employed for ego preservation as evidenced by his tendency to embellish his abilities before noting his deficiencies. This was noted when he discussed his eyesight, academic challenges, and TBI. Additionally, he demonstrated concerning judgment as evidenced by his expressed desire to increase his alcohol and cannabis use. It was noted that he often diminished his accountability regarding historical incidents when encountering problems with authority. Based on the information above, the following test results are considered an estimation of Mr. Nimer's psychological functioning, substance use, and mental state.

TEST RESULTS

Wechsler Adult Intelligence Scale - Fifth Edition (WAIS-V):

The WAIS-V was administered and scored according to the standardized procedures as outlined in the manual. It was noted that Mr. Nimer frequently asked if the subtests were timed and on subtests that were not timed, Mr. Nimer significantly longer than usual to provide an answer despite being prompted by the examiner. It was noted that Mr. Nimer denied taking any medication the morning of the present evaluation.

Mr. Nimer obtained a Full-Scale Intelligence Quotient (FSIQ) that placed him in the *Above Average* range of intellectual functioning (Composite Score of 119; 95% Confidence Interval Score was calculated to be between 113 and 123). The FSIQ is derived from a sum of 7 subtest scaled scores: Similarities, Vocabulary, Block Design, Matrix Reasoning, Figure Weights, Digit Sequencing, and Coding. The FSIQ is typically considered the score that is the most representative of general intellectual functioning.

Verbal Comprehension. Mr. Nimer's ability to access and apply acquired word knowledge as measured by the Verbal Comprehension Index (VCI) was in the *extremely high* range (VCI = 140; 95% confidence interval = 130-144). High VCI scores indicate a well-developed verbal reasoning system with strong word knowledge acquisition, effective retrieval, good ability to reason and solve verbal problems, and effective communication of knowledge. The VCI is derived from the sum of scaled scores for Similarities and Vocabulary. The VCI provides an estimate of crystallized intelligence. The application of this knowledge involves verbal concept formation, reasoning, and expression.

Visual Spatial. Mr. Nimer's ability to evaluate visual details and to understand visual-spatial relations to construct geometric designs from a model as measured by the Visual Spatial Index (VSI) was in the *above average* range (VSI = 113; 95% confidence interval = 105-119). High VSI scores indicate a well-developed capacity to apply spatial reasoning and analyze visual details. The VSI is derived from the sum of scaled scores for Block Design and Visual Puzzles. The ability to construct designs requires visual-spatial reasoning, integration and synthesis of part-whole relations, attentiveness to detail, and visual-motor integration.

Fluid Reasoning. Mr. Nimer's ability to detect the underlying conceptual relations among visual objects and to use reasoning to identify and apply rules as measured by the Fluid Reasoning Index (FRI) was in the *average* range (FRI = 100; 95% confidence interval = 93-107). His score indicates average ability to abstract conceptual information from visual details and to effectively apply that knowledge. The FRI is derived from the sum of scaled scores for Matrix Reasoning and Figure Weights. The FRI provides an estimate of fluid intelligence. Identification and application of conceptual relations in the FRI requires inductive and quantitative reasoning, broad visual intelligence, simultaneous processing, and abstract thinking.

Working Memory. Mr. Nimer's ability to register, maintain, and manipulate auditory information in conscious awareness was in the *average* range (WMI = 103; 95% confidence interval 96-110). His scores indicate an average ability identifying auditory information, maintaining and updating it in temporary storage, and resequencing it for use in problem solving. The WMI is derived from the sum of scaled scores for Digit Sequencing and Running Digits, which requires attention, auditory discrimination, and concentration.

Processing Speed. Mr. Nimer's speed and accuracy of visual identification, decision making, and decision implementation was in the *average* range (PSI = 102; 95% confidence interval 93-110). His scores indicate average abilities when rapidly identifying visual information, making fast and accurate decisions, and implementing those decisions quickly. The PSI is derived from the sum of scaled scores for Coding and Symbol Search. Performance on the PSI is related to visual scanning, visual discrimination, short-term visual memory, visual-motor coordination, and concentration. The subtests contributing to the PSI are not measures of simple reaction time or simple visual discrimination; a cognitive decision-making and learning component is involved.

WAIS-V Summary. Mr. Nimer is a 33-year-old male who completed the WAIS-V. His general cognitive ability, as estimated by the WAIS-V, is in the *above average* range (FSIQ =

119). His general verbal comprehension abilities were in the *extremely high* range (VCI = 140); his general understanding of visual-spatial relations were in the *above average* range (VSI = 113); his general ability to detect the underlying conceptual relations among visual objects were in the *average* range (FRI = 100); his general abilities to register, maintain, and manipulate auditory information in conscious awareness were in the *average* range (WMI = 103); and his general ability to use speed and accuracy of visual identification, decision making, and decision implementation was in the *average* range (PSI = 102).

Mr. Nimer's score on the VCI is considered a cognitive strength and his score on the FRI is considered a cognitive weakness relative to his overall performance. There was a significant difference noted in his VCI > FRI scores, which implies a strength in crystallized abilities relative to fluid reasoning abilities; VCI > VSI, which indicates a relative strength in using verbal stimulus in problem-solving compared to visual-spatial problem solving; VSI > FRI, implying stronger concrete visual-perceptual processing skills relative to fluid reasoning; VCI > WMI, indicating the relatively lower working memory ability may interfere with complex problem-solving; VCI > PSI suggesting that verbal comprehension is stronger than processing speed; and VSI > PSI, implying Mr. Nimer can solve complex problems despite having relatively lower processing speed ability. When evaluating subtest-level strengths and weaknesses, Similarities and Vocabulary were noted as strengths while Figure Weights and Running Digits were noted as weaknesses. Additionally, when evaluating pairwise comparisons, a discrepancy was noted between Matrix Reasoning and Figure Weights, implying a relative strength in inductive reasoning compared to quantitative reasoning. There was also a discrepancy between Digits Sequencing and Running Digits, indicating that he may best utilize working memory when performing active mental manipulation on the registered and maintained information rather than actively maintaining, refreshing, and updating information. It is also possible that a slower pace benefits his auditory registration of information.

Personality Assessment Inventory (PAI):

Mr. Nimer responded to all PAI test items. His responses on validity scales were all in appropriate ranges. As a result, Mr. Nimer's results are considered valid and interpretable, in conjunction with all additional available sources of information.

Clinical features. Mr. Nimer reports a personality style that involves a degree of adventurousness, risk-taking, and a tendency to be rather impulsive. Others may view him as pragmatic and perhaps unsympathetic in his relationships. At times his behavior is likely to be reckless; he can be expected to entertain risks that are potentially dangerous to himself and to those around him. He is likely to be easily bored by routine and convention, and he may act impulsively in an effort to stir up excitement.

Self-concept. Mr. Nimer's self-concept appears to involve a generally stable and positive self-evaluation. He is normally a confident and optimistic person who approaches life with a clear sense of purpose and distinct convictions. These characteristics are valuable in that they allow him to be resilient and adaptive in the face of most stressors. He describes being reasonably self-satisfied, with a well-articulated sense of who he is and what his goals are.

Interpersonal and social environment. Mr. Nimer's interpersonal style seems best characterized as involving strong needs for affiliation and positive regard from others. This may result in rather uninhibited social behavior that may be seen by others as attention-seeking and dramatic. These needs for attention and affiliation can be so strong that the quality of his social

interactions may be relatively unimportant as compared to their quantity. These behaviors, perhaps intended as friendly and sociable by him, might be viewed as somewhat overbearing by those around him. In considering the social environment of Mr. Nimer with respect to perceived stressors and the availability of social supports with which to deal with these stressors, his responses indicate that both his recent level of stress and his perceived level of social support are about average in comparison to normal adults. The reasonably low stress environment and the intact social support system are both favorable prognostic signs for future adjustment.

Treatment considerations. Mr. Nimer is not reporting distress from thoughts of self-harm. He describes himself as a very meek and unassertive person who has difficulty standing up for himself, even when assertiveness is warranted. Thus, he may have some difficulty in the appropriate expression of anger. Mr. Nimer's interest in and motivation for treatment is somewhat below average in comparison to adults who are not being seen in a therapeutic setting. Furthermore, his level of treatment motivation is substantially lower than is typical of individuals being seen in treatment settings. His responses suggest that he is satisfied with himself as he is, that he is not experiencing marked distress, and that, as a result, he sees little need for changes in his behavior. However, he does report a number of strengths that augur well for a relatively smooth treatment process if he made a commitment to treatment. If treatment were to be considered, particular areas of attention or concern in the early stages of treatment could include that he may not be experiencing sufficient distress to feel that treatment is warranted. He may be somewhat defensive and reluctant to discuss personal problems, and as such he may be at-risk for early termination.

Adult Substance Abuse Subtle Screening Inventory – 4th Edition (Adult SASSI-4):

Mr. Nimer was directed to respond to items that included his lifetime use of substances. His profile reflected no evidence of random responding and his score on the defensiveness scale displayed no evidence of a problem. His profile confirms a high probability of having a substance use disorder and a high risk of a problem of acting out. His profile indicates no evidence of emotional pain, or prescription drug abuse. It is recommended that the treatment plan include specific interventions directed toward the substance-related disorder. It is unlikely that education alone will adequately address his substance use problem. Treatment should include some form of addictions therapy (individual and/or group) and exposure to appropriate community support groups.

Mr. Nimer is at relatively high risk for legal problems and other types of norm violations. The problem is compounded by substance use. If he is using psychoactive substances, there may be an increased risk of impulse control problems and poor judgment. It is therefore recommended that the substance use treatment be highly structured and provide adequate support for comprehensive behavioral change. Structured, didactic, cognitive-behavioral interventions for both substance use disorder and impulse control problems will be most beneficial.

DIAGNOSTIC IMPRESSIONS AND CONCLUSIONS

Mr. Nimer's self-report, behavioral observations, mental status, records provided, and test results were reviewed in totality to determine clinical diagnostic impressions and conceptualization relating to the reason for the referral. Mr. Nimer presents with some ongoing neurocognitive concerns consistent with the most recent neuropsychological evaluation by Dr. Kerrigan, Ph.D., dated 07/04/2018, which stated that since Mr. Nimer's motor vehicle accident

on 02/07/2015, he sustained some deficits in executive functioning involving self-regulation, self-management, self-monitoring, task completion, and mental flexibility (some of which may be related to pre-existing ADHD, hyperactive type). However, Dr. Kerrigan's test data further suggested that Mr. Nimer has some sustained difficulties, which Dr. Kerrigan explained was commonly associated with impacts of head trauma despite not having symptoms that warranted a sustained diagnosis for a mild TBI.

Given Mr. Nimer's neurocognitive vulnerabilities reported above, and continued executive functioning concerns related to mental flexibility, self-regulation, and disinhibited behavior, a current neuropsychological evaluation is warranted to further evaluate Mr. Nimer's neurocognitive abilities and any residual impairments associated with his prior head injury. This evaluation should also assess the need for specific vocational accommodations, if deemed appropriate, highlighting personal strengths and functional limitations. A notable strength from this evaluation is Mr. Nimer's above-average intellectual functioning and extremely high verbal comprehension skills, which will serve him well in his vocation.

Substance use findings indicate some heightened risk for relapse on alcohol and cannabis, particularly to manage stress and life dissatisfaction. He self-disclosed a preference to resume cannabis and alcohol use on the weekends, suggesting poor insight into relapse risk and use of substances for possible stimulus seeking purposes.

DSM-5-TR Diagnostic Impressions:

Attention-Deficit/Hyperactivity Disorder, Predominantly hyperactive/impulsive presentation
(per medical records)

Alcohol Use Disorder, Moderate

Cannabis Use Disorder, Mild, In early remission

Recommendations:

Based on this evaluation, it is recommended that should the Attorney Admissions find that Mr. Nimer is eligible for conditional admission, that he be admitted to the practice of law under the following conditions:

- 1) Applicant obtain an updated neuropsychological evaluation to further evaluate neurocognitive abilities and any residual impairments associated with his prior head injury. This evaluation should also assess the need for specific vocational accommodations, if deemed appropriate, highlighting personal strengths and functional limitations. Prior neuropsychological evaluations, brain imaging results, and neurological records, including a copy of this report should be provided to the evaluator to assist in the evaluation.
- 2) Applicant continue psychiatric treatment with provider who has expertise in co-occurring substance use and mental health disorders (ADHD). It is important for monitoring of prescribed dependence-forming medications (stimulant), given Mr. Nimer's history and addiction risk potential.
- 3) Applicant engage in supportive outpatient program (SOP) for relapse prevention, increasing insight, effective coping skills, and address ongoing risk factors related to substance use. Recommended frequency and duration is approximately 2 group sessions a week for 8-12 weeks, or otherwise advised under the discretion of the treatment facility.

- 4) Applicant engage in psychotherapy with a minimum master's level mental health provider with expertise in dual-diagnosis treatment. Therapy focus areas are suggested to include relapse prevention, impulse control, emotional regulation, and executive dysfunction. Frequency and duration are recommended to be once a week for 3 months, then reduced to twice a month for 3 months, then once a month for 6 months (12 months total).
- 5) Applicant abstains from alcohol and drugs, unless prescribed by a qualified medical/psychiatric provider, with random alcohol screening for accountability and monitoring for a minimum of 6 months.
- 6) Applicant obtains a mentor with the Members Assistance Program (MAP) and makes monthly contact with the mentor for at least 12 months and causes the MAP mentor to provide quarterly compliance reports to the State Bar of Arizona for at least 12 months.
- 7) Applicant has the providers in #2, #3, and #4 above provide compliance reports to both MAP and the State Bar of Arizona once every 3 months for 12 months.
- 8) Applicant participates in a structured recovery program, such as attending 12-step recovery (Alcoholics Anonymous or equivalent) weekly.

If admitted, these are the conditions under which I recommend the Applicant should be monitored. I make no recommendations regarding whether Mr. Nimer should be admitted to the State Bar of Arizona. Any recommendations regarding the qualifications for admission are beyond the scope of this report. This evaluation and recommendations should be considered only after the Committee on Character and Fitness has reached a decision regarding the recommendation for admission of the Applicant.

Respectfully submitted,



Emily T. Bashah, Psy.D.

Licensed Psychologist, Arizona PSY-004816